

CITATION: Anand v. Belanger, 2010 ONSC 2619

COURT FILE NO.: 04-CV-266354CM1

DATE: 20100503

ONTARIO

SUPERIOR COURT OF JUSTICE

BETWEEN:

GEETA ANAND, DHARAM DEV
ANAND and SANDEEP ANAND

Plaintiffs

– and –

VALERIE BELANGER, and TRACY
TOLLAND AND STATE FARM MUTAL
AUTOMOBILE INSURANCE COMPANY

Defendants

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) Jeffrey William Strype, for the plaintiffs
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) Sandi J. Smith, for the defendant State Farm
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) HEARD: April 29, 2010
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STINSON J.

[1] This ruling concerns a “threshold” motion, that is, a motion brought by the defendant for an order dismissing this action on the ground that the plaintiff Geeta Anand did not sustain a permanent serious impairment of an important physical, mental or psychological function in the motor vehicle accident giving rise to this claim and thus the defendant has no liability by reason of s. 267.5(5) of the *Insurance Act*, R.S.O. 1990, c. I.8 as amended by S.O. 1996, c. 21, s. 29. The motion was argued after I concluded my charge to the jury and while the jury was deliberating. While I was deliberating on the motion submissions and before my ruling was prepared, the jury announced it had reached its verdict. With the approval of counsel, the jury returned and delivered its verdict on the express understanding of counsel that I would subsequently complete and deliver my ruling on the motion under reserve.

FACTUAL BACKGROUND

[2] This is an action seeking damages for a motor vehicle accident that occurred on April 26, 2003. The plaintiff Geeta Anand was a passenger in a motor vehicle owned and operated by her son, the plaintiff Sandeep Anand. She was injured when the Anand vehicle was struck from

behind by a vehicle owned by the defendant Valerie Belanger and driven by the defendant Tracey Tolland. Tolland had stolen the Belanger vehicle, as a consequence of which the Belanger vehicle was uninsured. Tolland was also uninsured.

[3] Sandeep Anand and his father Dev Anand (husband of the plaintiff Geeta Anand) were insured under policies of automobile insurance issued by the defendant State Farm Mutual Automobile Insurance Company. By reason of the uninsured status of the Belanger vehicle and its driver, Mrs. Anand was entitled to sue State Farm under the uninsured motorist provisions of the State Farm policies for her tort damages. The parties have agreed that Belanger bears no liability for this action, and therefore the action was discontinued as against her. Tolland was never served with the statement of claim and the action was discontinued as against him. Thus the only defendant at trial was State Farm, which stepped into the shoes of Tolland for purposes of defending the action.

[4] At the time of the accident, the Anand car was stopped at the red light when the Belanger vehicle collided with it at its rear right corner. As a result, the Anand vehicle was pushed forward into the intersection, but there was no secondary collision or impact. Approximately \$15,000 to \$20,000 damage was done to the Anand vehicle.

[5] Mrs. Anand was in the right rear passenger seat, wearing her seat belt. According to her evidence, she was seated on somewhat of an angle in the rear seat. This is understandable because she is a relatively large (240 lbs or so at the time) person, and the car was a compact 4 door sedan without much rear seat space. In the aftermath of the collision her whole body was "thrown about". Among other things, she says that her left knee (or her knees) hit the rear of the front seat. She was unable to get out of the car unaided. When the paramedics came, they put on a neck collar. Her whole body was in pain, mostly her neck, back and left knee. She was taken to hospital but not admitted due to the then SARS crisis, and was discharged home. According to Mrs. Anand, she stayed in bed lying down most of the next day, Sunday, April 27, but when she tried to get up, her left leg would not move: it was swollen and bruised in colour. She also had lower back pain and could not sit. She went to see her family doctor, Dr. Lerner, on Monday, April 28 and was referred to a physiotherapist.

[6] Up until the day of the accident, Mrs. Anand had worked for approximately 15 years in an unskilled manual labour position at a packaging factory; indeed, she worked a shift on Saturday, April 26, the day of the accident. She frequently worked long hours and accumulated considerable overtime and shift bonuses. Her work was physically demanding, in the sense that it involved prolonged periods of standing, moving and lifting.

[7] Mrs. Anand has not worked since the accident, despite her stated desire to return to work. She says she loved her job, but was unable to return to work because of the pain she experienced subsequent to the accident, ranging from pain in her neck and shoulders to her back and in particular her left knee. She says she is no longer able to stand or walk for any period of time, and she walks with a limp using the cane.

[8] She attributes her physical problems to the accident. Although there were some mentions in her medical records of intermittent complaints of knee pain before the accident (mostly her right knee) she testified that her disabilities did not arise until after the car accident. She says that as a result of the injuries she suffered in the accident, she has been unable to continue to work and has been unable to continue to do household chores and cook and clean as she formerly did. The injuries have caused her significant pain and have had a severe negative impact on her overall physical and mental health, her social relationships and her relationships with her family. Among other diagnoses, she is said to have knee degeneration in her left knee to the point that she requires a knee replacement, fibromyalgia or chronic pain, and a chronic major depressive order.

[9] At the heart of the case is the question whether the motor vehicle accident caused the medical problems of which Mrs. Anand now complains. In very general terms, the plaintiff's position is that Mrs. Anand suffered soft tissue injuries along trauma to the knee (including a partial tear to the anterior cruciate ligament) in the motor vehicle accident, which has progressed to debilitating knee and other problems and chronic pain. The defendant's position is that the plaintiff's problems were pre-existing, were not caused by the accident and in any event have been worsened by her excessive weight. Thus, the defendant denies that Mrs. Anand has any permanent injuries arising from the accident.

ANALYSIS

[10] Pursuant to s. 267.5(5) of the *Insurance Act*, the defendant is not liable unless Mrs. Anand has sustained "permanent serious impairment of an important physical, mental or psychological function" as a result of the motor vehicle accident. This motion is therefore brought under s. 267.5(15), which requires me to make that determination. The onus is on the plaintiff.

[11] Although the defendant did not concede that Mrs. Anand's current medical condition is such that it amounts to a "permanent serious impairment of an important physical, mental or psychological function", the true focus of the argument was whether her condition was caused by the accident or other sources, namely, pre-existing and inevitable degenerative osteoarthritis that took the natural and ordinary course and toll on her knees that it would have regardless of the accident. The causation issue requires a consideration of both the lay evidence and that of treating and expert witness health professionals who testified about their observations of Mrs. Anand.

Lay Evidence

Mrs. Geeta Anand

[12] The thrust of Mrs. Anand's evidence is that she was able to function at work and at home before the accident, but as a result of the injuries she suffered in the car accident she has been unable to continue to work and been unable to continue to do household chores and cook and

clean as she formerly did. The injuries have caused her significant pain and have also had a severe negative impact on her overall physical and mental health, on her social relationships and her relationships with her husband, children and grandchildren. According to Mrs. Anand she continues to suffer significant pain. She uses a cane but walks with a distinct limp and gait. I had the opportunity to observe her first hand, and it was apparent to me that she was in discomfort as she moved about the courtroom and as she sat in the witness chair. Her physical appearance in front of me in court was consistent with the testimony of her family members regarding her condition and their observations of her.

[13] As mentioned above, Mrs. Anand testified that she hit her knee on the seat ahead of her, resulting in pain, bruising and swelling. Her family doctor's records do not record her as having complained to him immediately about her knee, and make no mention of bruising or swelling in that area until several visits later. Despite that omission, Mrs. Anand does appear to have suffered some injury to her left knee at or around this time, since bruising to her left knee was reported by the physiotherapist a few days after the accident. That said, Mrs. Anand described the bruising to the knee in far more dramatic terms than the physiotherapist. It may be that Mrs. Anand's recollection is based on her subsequent experience, as the swelling and bruising became more material.

[14] I would observe that Mrs. Anand is not the most reliable historian. This may in part be attributable to the fact that the initial events in question took place over 7 years ago, and in the succeeding years she has seen many doctors for various examinations as a consequence of which she has difficulty in recalling what she said to whom and when. She also has difficulty remembering events that took place before the accident, but once again, this may be attributable to the lengthy passage of time.

[15] I am also conscious that Mrs. Anand does not speak English as a first or even a second language. She testified through an interpreter, which might explain why some of her answers did not appear to be fully responsive to the questions posed. It may also have been a factor in her communications with doctors and other examiners over the years.

[16] Mrs. Anand also sometimes gave information that was contradicted by the written record. An example of this was her evidence concerning her weight at the time of the accident (a claimed 200 to 210 lbs. by her versus 245 lbs on the physiotherapy intake assessment). Again, in light of the evidence that Mrs. Anand's weight has varied considerably (both up and down) since the accident, I would not be terribly critical about that misstatement

[17] I further acknowledge that Mrs. Anand sometimes exaggerated. One example was her evidence regarding the size of the bruise on her knee subsequent to the accident. She said that the knee was black and blue and the bruise was about the same size as her hand. By contrast, the physiotherapist's examination suggested that it was a small bruise about the size of a quarter. This may be explainable by the fact that, according to her, the swelling and bruising continued in the weeks after the incident, however.

[18] There were also conflicts between her description of how much work she could do at home in September 2003 and those contained in the physiotherapy assessment report and the evidence of Kylie James. Once again, however, this may merely reflect Mrs. Anand's inability to recall with specificity her physical state at a particular point in time, given that she is now looking back some 7 years at an ongoing series of problems. It is also, however, consistent with the knee improving for a period of time until further stress on it created additional difficulties, which led to her MRI scan in January 2004 and her knee surgery in June 2004.

[19] Overall, despite these admitted flaws in Mrs. Anand's testimony, I found her evidence largely credible. In particular, her evidence concerning the mechanism of the injury to her knee is consistent with the expert testimony of Dr. Karabatsos (which, as described below, I accept). It is also supported by the evidence of her family members, physiotherapist and family physician as regards the bruising and her mobility difficulties following the accident. Further, it is unarguable that she was virtually asymptomatic as far as any knee problems were concerned up until the accident, able to work a full shift that very day, yet unable to return to work since. This is fairly powerful evidence that the collision resulted in a serious injury.

Dev Anand

[20] Dev is the husband of the Plaintiff. He testified about their life prior to the accident and how Geeta had no significant medical problems prior to the accident. Dev also testified about the accident and its aftermath, since he was the front seat passenger before the collision took place. He described the bruising and swelling on her knee as of the next day and the pain and associated problems that followed. In general terms, he described the difficulties that his wife has encountered and how they have increased, and the limitations that have been imposed on her activities both inside and outside the home as a result.

[21] Mr. Anand testified in a straight-forward fashion and was not seriously challenged on cross-examination. He impressed me as a witness who tried to give fair and full answers to the questions posed. His evidence was largely internally consistent, although he was a little unclear about when he and his wife had started sleeping in separate bedrooms. He is obviously wholly allied with the plaintiffs' case, but I found him to be a largely credible witness.

Deepika Marula

[22] Deepika is the daughter of Geeta and Dev and sister of Sandeep. She testified about her mom's general good health prior to the accident and her poor health subsequent to the event, indicating that her mother no longer has the physical capability that she formerly had. Deepika is obviously strongly allied with the plaintiff's side. That said, her evidence was relatively uncontroversial and there is no reason to significantly doubt her credibility.

Sandeep Anand

[23] Sandeep is Dev and Geeta's son and was driver of the car at the time of the accident. In addition to describing the accident and the efforts taken to extract his mother from the car,

Sandeep described the previous activities of his mother and her post-accident limitations. In this respect his evidence was consistent with that of Geeta and Dev. His observation regarding the bruise and swelling to his mother's left knee is consistent with the observation recorded in the initial assessment report of the physiotherapist.

Anju Anand

[24] This is the Plaintiff's daughter-in-law, married to Sandeep. She also confirmed noticing the bruising on the Plaintiff's left knee several days after the accident.

Review of Medical Evidence

[25] Many professional witnesses gave evidence concerning Mrs. Anand's condition. Some were treating health professionals while others were consultants who were retained to assess Mrs. Anand and give their professional opinion concerning her problems for purposes of this lawsuit. In an attempt to put their evidence in some logical order, I will refer to them in the chronological sequence in which they met with and assessed Mrs. Anand. I will not attempt to recite the evidence of all, but just the principal ones.

Dr. Lawrence Lerner

[26] Dr. Lerner was Mrs. Anand's family physician from 1989 through the present. His records reflect Mrs. Anand's various ailments and his diagnosis and treatment recommendations over the years. He is a general practitioner, not a knee specialist, and he testified as a fact witness, not as an expert or opinion witness.

[27] Mrs. Anand's initial visit to Dr Lerner back in 1989 concerned a complaint of left knee pain, although there was only one further such complaint over the years prior to the accident. Dr. Lerner treated a variety of right knee and other complaints from time to time, and suggested a diagnosis of osteoarthritis. The X-rays taken prior to the accident, however, never showed osteoarthritis. Dr. Lerner was firmly of the view that because Mrs. Anand functioned satisfactorily prior to her accident and encountered such difficulties after the accident, that the problem must be accident-based. Dr. Lerner's evidence is that he disagreed that Mrs. Anand was fit to return to work. He based his assessment on his overall knowledge of her condition, as a long time patient. As I have noted, however, he is a general practitioner and did not testify as an expert in musculoskeletal problems.

[28] Another point worth noting is that over the years Dr. Lerner frequently prescribed anti-inflammatories to the Plaintiff, giving rise to the suggestion that she may have had more difficulties with her knees than his records reveal and that these were kept in check by the anti-inflammatories he was prescribing for her other ailments.

[29] Dr. Lerner is sympathetic to the Plaintiff, as her long time family physician. He has seen her through many difficulties, particularly over the last 7 years. Dr. Lerner has a very busy practice, by his own account, and he claims this explains why his notes are not as complete as

they should be. He maintains, for example, that Mrs. Anand did have knee pain problems immediately following the accident, although they were secondary which is why he did not record them in his notes. He also claims to recall swelling and bruising to her left knee, something also not recorded in his notes. The absence of mention of this particular feature of Mrs. Anand's post-accident condition might ordinarily raise questions as to the accuracy or reliability of this aspect of Dr. Lerner's testimony. Significantly, however, the original assessment performed by the CARE physiotherapy clinic records that Mrs. Anand had bruising and swelling to her left knee. The physiotherapy assessment was performed on April 30, 2003, only 4 days after the accident. This would suggest that, despite the absence of this information in his records, Dr. Lerner may well have observed the condition. Presumably the swelling continued until the physiotherapy examination. The presence or absence of any complaint of left knee pain, bruising or swelling was not an issue at the time, and thus the information about bruising and swelling and pain in the left knee contained in the physiotherapy assessment report is corroborative of the Plaintiff's position and to a certain extent goes to fill in the gap in Dr. Lerner's notes.

Barbara Zaccai

[30] Ms. Zaccai was the treating physiotherapist to whom the Plaintiff was referred by Dr. Lerner. She first saw Mrs. Anand in April 2003, a matter of days following the accident. Although the initial assessment mentioned some bruising to the knee and some pain there, the principal focus of the early assessment and treatment was the whiplash injury. Over time (between May and August 2003) the knee pain and problem seemed to be resolving, but when Mrs. Anand put more stress on her knee the problem got worst. By the fall of 2003, it was apparent that the knee problem was worsening and that it was not healing as expected. As a result, Mrs. Anand was referred to an orthopedic surgeon to perform an arthroscopic procedure. She returned for further treatment in the summer of 2004 but she still had 8 out of 10 pain in the left knee.

[31] During the initial treatments by Ms. Zaccai, Mrs. Anand's weight increased by at least 20 pounds; upon her return for treatment, however, her weight was down almost 50 pounds.

[32] Based on Ms. Zaccai's evidence, it would appear that there was not as much bruising on the knee as the Plaintiff described (at least initially) but some form of injury was apparent. The swelling increased over time and the knee did not heal as expected. As a result, the initial physiotherapy was discontinued, surgery performed and follow up physiotherapy pursued. This evidence, therefore, would be consistent with an injury or aggravation of the problem resulting from the car accident, that led to subsequent, significant limitations in the Plaintiff's mobility due to the knee problem.

[33] Although Ms. Zaccai was paid professionally to attend, her evidence was, in my view, completely objective. She was careful and precise and responsive to questions, both in chief and on cross-examination. She explained as appropriate and her evidence was internally consistent. I would rate her credibility as high.

MRI Report

[34] Mrs. Anand underwent an MRI scan of her left knee joint in January 2004. The stated conclusion was that she had a degenerative joint with a degenerative type tear of the posterior horn of the medial meniscus and a partial tear of the anterior cruciate ligament.

Dr. Ed Urovitz

[35] Dr. Urovitz was the orthopedic specialist to whom Mrs. Anand was referred in the Fall of 2003. He ultimately performed the arthroscopic knee surgery that was carried out in June 2004. Although he was not called to testify, his records were filed as an exhibit. According to his report of the operation, when he performed the surgery, he did not define a tear in her medial meniscus, but found fraying around the edge, which he trimmed. His records say nothing about a partial tear of the ACL.

Dr. J. Ogilvie-Harris

[36] Dr. Ogilvie-Harris is an orthopedic surgeon who examined Mrs. Anand on a medical-legal referral basis on behalf of the Plaintiffs in July 2005. His expert report was filed. It was his opinion that, in the motor vehicle accident, Mrs. Anand suffered soft tissue injuries to her cervical and lumbar spine, resulting in damage to the muscles and ligaments, with ongoing long-term problems. He also stated that Mrs. Anand sustained an injury to her left knee; although she had pre-existing degenerative changes, she had no substantial symptoms pre-accident, and could have continued for many years without significant complications or deterioration. His evidence thus supports the plaintiffs' theory that Mrs. Anand's problems were caused by the car accident.

Dr. Edward English

[37] Dr English testified for the defence as an expert in the field of orthopedic surgery. He examined Mrs. Anand in October 2005. He observed that she had difficulty walking and was quite dependent on her cane. He took a full history and conducted a physical examination. He found she had full movement of her knees and no significant laxity in her knee ligaments. He did note she had some wasting of her quadriceps muscle of her left leg. He also reviewed Mrs. Anand's medical records.

[38] In relation Dr Urovitz's report on his arthroscopy on Mrs. Anand's left knee, Dr. English noted that it contained no comment about a possible partial tear to the anterior cruciate ligament. He went on to add, however, that arthroscopic inspections are not 100% guaranteed and that there is a 10% chance that something will be missed, thus leaving open the possibility that there may have been damage that was not detected by Dr. Urovitz.. He opined that the ACL partial tear mentioned in the MRI report was not attributable to the accident, although he offered no alternate explanation for how it could have occurred.

[39] On cross-examination he agreed that such an injury can be caused by a frontal impact to the knee, such as the one experienced by Mrs. Anand in the accident. He also agreed that that

swelling and bruising to the knee (such as that recorded in the notes of the physiotherapist) were symptoms of a serious injury. He went on to state in re-examination that the result of trauma to the ACL depends on the person, but if someone is able to walk around, there should be no acute injury to the knee. He added that he did not think that Mrs. Anand's knee problems were caused by any ACL instability, but rather were the result of the onset of pre-existing osteoarthritis that Mrs. Anand became aware of after the accident.

[40] While he is a long-time orthopedic surgeon, Dr. Lawson's specialty is in feet and ankles, not knees. While I have no reason to question his expertise, he did not have a ready explanation for the sudden change in the plaintiff's functioning pre- and post-accident, save to say that the accident caused her to become aware of her knee problems. He finished his re-examination by saying that Mrs. Anand suffered from the gradual onset of arthritis, something I have difficulty reconciling with the sudden change in her abilities. I did not find his explanation persuasive.

Dr. Gordon Lawson

[41] Dr. Lawson saw Mrs. Anand on a medical legal referral for the Plaintiffs in February 2009. He is a chiropractor with expertise in sports injuries and rehabilitation. He works with Dr. Ko, a physiatrist, and they did a collaborative assessment of the Plaintiff's condition. He identified a diagram showing a representation of the Plaintiff's left knee based on the MRI report, revealing a tear in the anterior cruciate ligament and a tear of the posterior portion of the medial meniscus, together with degenerative changes in the meniscus and the femoral condyle.

[42] It was Dr. Lawson's opinion that these problems were related to the motor vehicle accident. He based his opinion on Mrs. Anand's report that she struck her knee on the console of the vehicle and his review of her self-reporting problems and the other medical examinations. He conceded that there had been self-limiting behaviour demonstrated during the functional assessment evaluation and that she had some abilities. The centerpiece of his opinion appears to be the absence of any significant limitations prior to the accident and the onset of limitations after it, which is factually supported by the lay witnesses and in particular by Mrs. Anand's work history.

[43] Dr. Lawson presented as an experienced and careful expert witness. He gave his evidence in a straight-forward fashion and, for the most part, fairly conceded points that were put to him on cross-examination that were not helpful to the plaintiff's case. He did, however, from time to time add additional information to explain or qualify some answers in order to limit the impact of some information adduced by the cross-examiner. That said, he did not do so in an aggressive or advocative fashion, in my view. He appeared to be relatively objective, albeit supportive of the plaintiff's position.

Dr. Gordon Ko

[44] Dr. Ko was the medical legal physiatrist expert for the Plaintiffs, who worked with Dr. Lawson in preparing an assessment report. He, too, saw Mrs. Anand in February 2009. It was his

opinion that the motor vehicle accident was the "tipping point" that resulted in the Plaintiff suffering the symptoms of which she complains today. He also diagnosed her with having fibromyalgia or chronic pain. While conceding that many of the responses to questionnaires and other tests that underlined the diagnosis of fibromyalgia involved subjective complaints, Dr. Ko was firm in his view that the Plaintiff suffered from this condition, based on his overall assessment and observations of the patient.

[45] Significantly, in his opinion, the tear in the horn of the cartilage does not happen spontaneously: it has to be the result of some trauma because it is not seen normally. This is a condition that does not heal and thus is consistent with some trauma leading to increased difficulty and additional degenerative problems with the knee.

[46] This witness has significant experience and expertise in the field of sports injuries and chronic pain and his opinion is entitled to considerable respect. Medical legal work is only a small percentage of his work, which is primarily treating persons with these conditions. He gave his evidence in a straight-forward, logical and persuasive fashion; on cross-examination he conceded points without argument. The cross-examination did not reveal any significant flaws in his analysis. While recognizing that he was retained to provide expert evidence that would assist the plaintiff, I would rate his credibility as very high.

Dr. Bob Karabatsos

[47] 88. Dr. Karabatsos is an orthopedic surgeon who examined Mrs. Anand on a medical-legal referral for the Plaintiffs in April 2009. He testified as an expert in knee surgery and conditions affecting the knee, not as a treating physician. He reviewed Mrs. Anand's medical records and, noting she had complained from time to time regarding her knees (in particular, her right knee) commented that mere knee pain from time to time does not mean an ongoing condition. He said that if Mrs. Anand had osteoarthritis she would have had trouble standing 8 to 12 hours per day at her job.

[48] The ongoing pain that was experienced by Mrs. Anand after the accident, he said, warranted further investigation, since a minor knee injury would have recovered by 6 weeks to 6 months. Based on his understanding of Mrs. Anand's medical history, her condition before and after the accident, her ongoing pain and the results of the MRI, Dr. Karabatsos formed the opinion that Mrs. Anand had suffered a traumatic injury to her left knee in the accident; in turn, this led to abnormal biomechanics of the knee, resulting in increased stress to the cartilage, and deterioration and arthritis. He testified that Mrs. Anand's main complaint to him was based on advanced arthritis. She will need a knee replacement, which is fairly major surgery. He agreed that arthritis can occur without trauma, and while it may not show up on an x-ray, if the symptoms are serious, the x-ray would reveal changes in the knee alignment. He further agreed that extra body weight can make the degenerative problem worse.

[49] Dr. Karabatsos is a knee specialist with an active surgical practice. His testimony was delivered in a calm, direct fashion, with suitable and appropriate illustrations and explanations.

His testimony was internally consistent and he was not seriously challenged on cross-examination. Overall, his evidence made sense and was consistent with the symptoms reported by Mrs. Anand over the years. It had a ring of truth and I found it very persuasive.

Dr. Judith Pialowsky

[50] This witness testified as an expert in the field of psychovocational assessments. She examined Mrs. Anand on behalf of the Plaintiffs in April 2009. She applied a variety of tests and reached the conclusion that Mrs. Anand suffers from a chronic major depressive disorder which arose because she was anxious, depressed and suffered pain. This witness believes that the accident affected the Plaintiff both in terms of anxiety and fear (as evidenced by her vomiting at the hospital) and the physical pain that she has subsequently experienced. The Plaintiff's problems are likely to get worse as her pain increases over time while she ages.

[51] Dr. Pialowsky was a very impressive witness. She gave careful, reasoned and responsive answers. Her evidence was internally consistent and made sense. She was careful to limit the scope of her testimony to matters within her own expertise and expressly qualified any comments she might make that were beyond her specific specialty. I found her to be highly credible.

Dr. Rajka Suric

[52] Dr. Suric is a physiatrist who was retained by the Defendant to conduct a medical legal assessment concerning Mrs. Anand's condition. In addition to her work as a treating physiatrist, Dr. Suric conceded that the majority of her income is derived from her work for insurers such as the Defendant, although that work takes up a minority of her time.

[53] Dr. Soric testified that she saw Mrs. Anand in April 2009, but had no record of using an interpreter in conversing with her. Mrs. Anand's major complaint was her worsening ability to walk, due to constant left knee pain. Cortisone injections and an arthroscopy had temporarily alleviated her knee pain. She also complained of low back pain, some neck pain, pain in her right arm and swelling in her right hand. Dr Soric said that Mrs. Anand did not indicate to her that she was depressed. She did note that Mrs. Anand walked with a significant limp and used a cane. On her physical examination, Dr Soric found no positive trigger points suggestive of fibromyalgia. She was quite dismissive (in my view, needlessly) of Dr. Kos' approach in using an algometer to test for chronic pain

[54] Dr. Soric's opinion was that Mrs. Anand had degenerative arthritis in her knees for a number of years prior to the accident. In her evidence in chief, she opined that Mrs. Anand suffered some pain due to a blunt trauma in the car accident, but no additional damage. Under cross-examination, she agreed that trauma could affect the knee, by causing a torn ACL, which could lead to degenerative changes.

[55] I am compelled to observe that Dr. Soric did not impress me as an expert witness. Quite apart from her long-time (and remunerative) involvement with defence insurers, during the

course of her testimony she frequently descended from the role of opinion witness to that of advocate, by either debating issues with counsel, or giving non-responsive answers, or providing information not sought by the questioner but (apparently) supportive of her theory. I am therefore not prepared to place much weight on Dr. Soric's evidence or opinion.

FINDINGS

[56] Overall, I found the plaintiff's evidence regarding the mechanism of her injury and the explanation given by her expert witnesses more persuasive than the evidence adduced on behalf of the defendant. In particular, I prefer and accept the evidence and opinions of Drs. Karabatsos, Ko, Ogilvie-Harris and Lawson, over that of Drs. Soric and English, for the reasons referable to the credibility of each as recounted above. In particular, the defence evidence failed to explain in a credible fashion why such a high-functioning person could be transformed in short order, effectively, into an invalid, in the absence of a material injury such as one suffered in the car accident. The plaintiff's case did explain this, both by objective evidence (the MRI scan report) and the expert evidence of Drs. Karabatsos, Ko, Lawson and Ogilvie-Harris, which I found persuasive, and which I accept.

[57] Even assuming that Mrs. Anand had some osteoarthritis prior to the accident, it was asymptomatic and did not interfere with her ability to work and otherwise function fully. As a result of the knee injury she suffered in the accident, the biomechanics of her legs was altered and her current problems flowed as a consequence,

[58] I therefore conclude that the plaintiff's current medical condition is a result of the motor vehicle accident of April 26, 2003. On the evidence before me I have no hesitation in concluding that the limitations on Mrs. Anand's mobility due to the deterioration of her knees by reason of the injury to her left knee in the accident are permanent, as is her fibromyalgia or chronic pain that also has its origins in the accident. Mrs. Anand's problems are ongoing and she is not going to recover her old level of mobility. There is no evidence that the chronic pain will be alleviated by possible knee replacement surgery. There can be no argument against the proposition that Mrs. Anand's mobility is an important bodily function. There can likewise be no dispute that her mobility limitations are serious.

DISPOSITION

[59] For these reasons, I hold that the plaintiff sustained a "permanent serious impairment of an important physical, mental or psychological function" as a result of the motor vehicle accident of April 26, 2003. She has therefore brought herself within the statutory exception to s. 267.5. The motion is therefore dismissed.

Stinson J.

Released: May 3, 2010

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Defendants

**REASONS FOR RULING
ON THRESHOLD MOTION**

Stinson J.

Released: May 3, 2010